

JUSTIFICATION AND APPROVAL FOR OTHER THAN FULL AND OPEN COMPETITION

Completion Instructions: The Department of Health and Human Services (HHS) has established a standard template for the preparation of a J&A for Other Than Full and Open Competition. If this is an 8(a) set-aside, please note this form is not required unless the acquisition is valued (including all options) at greater than \$22 million. This template is NOT applicable for actions using simplified acquisition procedures (FAR Part 13), Federal Supply Schedule orders (FAR 8.4) or task or delivery orders subject to fair opportunity (FAR 16.505).

Tailor all aspects of this template to the individual acquisition. Consult [FAR Subpart 6.3](#) for guidance. An addendum may be added in case additional space is needed.

When this document is printed, the blue italic instructions are automatically deleted on the printed version.

Acquisition Title:	Hospital Accreditation Compliance Assessments and Leadership Support
Agency:	IHS
Acquisition Year (FY):	2025
Author and Title:	Darryl Smith

2. Description of Action:

Nature:	☒ New Requirement	☐ Follow-on Requirement	
	☐ Modification to Existing Purchase Order/Contract Number:		
Pricing:	☒ Firm-Fixed Price	☒ Time & Materials	Cost \$2,853,132.00
	☐ Other:		
Funds:	☐ OMA	☐ Other Funds:	

Name of Proposed Contractor(s): Bryant Healthcare Consultants, Inc.

Street Address: 16 Old Stage RD

City, State, Zip: Hampton Falls, New Hampshire 03844-2021

3. Description of Services or Supplies:

Basis for Approval (FAR 6.303-1(d)):	☒ Individual Basis	☐ Class Basis
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The total estimated value of the proposed action, including all options is \$ 5,673,180.00

As applicable, briefly describe the services or supplies required to meet the agency's needs, including make & model number where appropriate. Include quantities of supplies or period of performance for services. Provide information for any options included. If the action is a modification to an existing contract, distinguish clearly between the work covered by the original contract and the work to be obtained by the proposed modification.

This service contract will provide consultation and professional support for the Office of Quality (OQ) to obtain expertise in the areas of hospital leadership mentoring, including the executive team (C-suite), support maintenance of The Joint Commission (TJC) accreditation standards and elements of performance (EPs) and Centers for Medicare and Medicaid Services (CMS) Conditions of Participation (CoP) within the Indian Health Service (IHS) system, and provide development of organizational gap analysis to improve and sustain hospital accreditation and compliance with CoPs, including, but not limited to leadership and governance, infection prevention and control,

patient safety, plant facility safety (environment of care and life safety standards), credentialing and privileging, nursing, laboratory, radiology, dietary, and Quality Assessment and Performance Improvement (QAPI) functions, according to accreditation standards and EPs and CoPs (further referred to as accreditation support).

4. Authority and Rationale: Identify the statutory authority, FAR title and FAR citation permitting other than full and open competition. It may be one of the following most commonly used citations by the operating divisions of the Department of Health and Human Services (HHS), but other exceptions may apply per [FAR Subpart 6.3](#).

Actions other than simplified acquisition procedures (select only one and provide an explanation):

☐ **FAR 6.302-1:** Only one responsible source and no other supplies or services will satisfy agency requirements, 41 U.S.C. 3304(a)(1)

Explain why the intended contractor is the only responsible source who can provide the required supplies or services. Discuss the unique capabilities, expertise, etc. that support the lack of competition/why no other type of supplies or services will satisfy agency requirements. Explain factors/unique qualifications such as proprietary data or exclusive licensing rights, if applicable. When competition is limited to items particular to one manufacturer, this justification must explain why the particular brand name, product, or feature is essential to the Government's requirements, and that market research indicates other companies' similar products, or products lacking the particular feature, do not meet, or cannot be modified to meet the agency's needs in accordance with FAR 11.105. If in connection with a follow-on contract for continued development or production of highly specialized equipment, detail the substantial duplication of cost or unacceptable delays.

☒ **FAR 6.302-2:** Unusual and compelling urgency, 41 U.S.C. 3304(a)(2)

If unusual and compelling urgency is the basis for the justification, explain the chronological events leading up to the requirement and explain why time constraints cannot permit even a limited competition. Describe the detrimental effects/serious injury to the mission of the requiring activity or to the government, financial or otherwise, that will result if this justification is not approved. Describe impact of required delivery/performance date. Describe the detrimental effects/serious injury to the mission of the requiring activity or to the government, financial or otherwise, that will result if this justification is not approved and the product or service cannot be provided by the intended sole source contractor. Failure to plan for expiring funds is not a valid reason for citing this exception. May not exceed one year, including all options, unless the head of the agency determines that exceptional circumstances apply.

Currently two (2) Indian Health Service (IHS) healthcare facilities are in jeopardy of being non-compliant with CMS Conditions of Participation (CoP) and accrediting body regulatory requirements. The anticipated period of performance will be for (6) months with consideration of adding one (1) six (6) month option period. Support is necessary to address the multitude of survey findings and recommendations in an urgent manner to ensure ongoing accreditation and reduce risk to our patients and staff. Should IHS not maintain quality medical services, these facility's ability to receive Medicare and Medicaid reimbursements could be lost, putting each facility in a precarious position with a large loss in revenue that supports increased access to medical service levels for their population served in these remote, isolated healthcare locales. Additionally, without support for the hospital executive leadership teams to develop the skills necessary to work through the many challenges they are facing, each facility is at great risk of losing even more exec team members imminently, which further puts continuity of operations at-risk. IHS facility are at risk for non-compliance for CMS conditions of

participation. Urgent and compelling to mitigate noncompliance-related to Medicare Conditions of Participation under the standard level deficiency related to physical plant and environment for providing safe care.

5. Agency Actions to Take or Remove Barriers That Led to Other Than Full and Open Competition (If applicable):

Describe the actions, if any, the agency may take to remove or overcome any barriers that led to the restricted consideration prior to any subsequent acquisition for the supplies of services is made. If no such actions have been taken, explain why.

IHS issued a Request for Information on SAM.gov 1/29/2025-2/12/2025; 12 responses were received from large and small businesses. However, none of the companies demonstrated capability to perform services required. In addition, a Sources Sought Notice was posted on SAM.gov 3/19/2025-3/29/2025; 8 responses were received from large and small businesses. However, none of the companies demonstrated capability to perform the services required and to meet the timeframe of services to begin no later than May 15, 2025 at all two locations. This posting can be found on SAM.gov under notice ID: IHS-25-SS-HCLMSS. Because the need for maintaining compliance is immediate at two IHS direct service hospitals, this short term contract will ensure no disruption to accreditation and ensure patients of IHS receive quality and safe healthcare. It is anticipated the current sources sought notice will result in a multi-award contract for which no barriers to competition are known or anticipated.

6. Bridge Contracts:

For contract extensions or bridge contracts when a competitive follow-on is in the process of being developed, summarize history of current contract and explain the reasons for any delays in the acquisition. Include discussion of the milestones for the follow-on action; the milestones should be as efficient as possible. Discuss why it would be neither cost effective nor realistic to expect another contractor to perform during the brief interim period; include issues such as start-up costs, phase-in, transfer of GFP, recruitment and staffing, etc. If the action is because of a protest, provide a brief discussion of the protest including the date the protest was filed and the basis of the protest. Explain that the action will provide the minimum quantity or performance period.

N/A

7. Actions to Increase Competition:

Describe efforts made to compete the action, including whether a notice was or will be publicized as required by FAR Subpart 5.2 or which exception under FAR 5.202 applies. If a notice was publicized, discuss the number of written responses to the synopsis and the results of the assessment of the written responses. List sources, if any, that expressed, in writing, an interest in the acquisition. Provide rationale if interested sources were rejected. If applicable, state that no other sources have expressed interest, but all offers received shall be considered. Describe what actions will be taken to increase competition before subsequent acquisition of the supplies or services is required. You may state that an action is a one-time requirement, but if a similar requirement arises, every effort will be made to compete it to the maximum extent possible. If action is sole source because of proprietary data or licensing rights and there will be a continuing need for the requirement, there must be evidence that advanced planning has been initiated to overcome the barriers to competition, or there should be documentation to support an analysis that replacement costs/licensing purchase costs outweigh the benefits of competition. If the action is a contract extension or bridge, explain actions to compete the follow-on. If action is an urgent new requirement and a competitive follow-on is anticipated, explain that efforts are underway to facilitate a full and open competition.

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8. Market Research:

Describe the extent of the market research conducted to identify all qualified sources and the results thereof. "Market Research" is defined as those attempts you made to ascertain whether other qualified sources exist, and can include contact with knowledgeable experts regarding similar or duplicate requirements, contact with industry, results of a sources sought synopsis, or draft solicitations. Research of the marketplace may consist of written, electronic (i.e. email) telephonic, or world wide web inquiries. Lack of advanced planning is not an acceptable reason for the lack of market research, but true urgency may necessitate abbreviated market research that is limited to readily available historical and commercial information. If action is a contract extension or bridge contract, describe the market research efforts underway or completed for the follow-on. See FAR 10.002(b)(2).

IHS issued a Request for Information on SAM.gov 1/29/2025-2/12/2025; 12 responses were received from large and small businesses. However, none of the companies demonstrated capability to perform services required. In addition, a Sources Sought Notice was posted on SAM.gov 3/19/25-3/29/2025; 8 responses were received from large and small businesses. However, none of the companies demonstrated capability to perform the services required and to meet the time frame of services to begin no later than May 15, 2025 at all two locations.

The Joint Commission Resources previously was contracted in 2017 to provide several IHS Great Plains Area hospitals with The Joint Commission Accreditation preparedness support. However, this vendor is not currently performing these services under contract at any IHS facility, and also IHS understanding is that this vendor is not capable of doing onsite day-to day engagements to address corrective actions recommended for adoption/implementation. In addition, while Tribal Health and Prime Physicians have expressed interest in this requirement, they are providing staffing services to several IHS hospitals that are at-risk of CMS and/or TJC accreditation non-compliance. Therefore, it would be a conflict of interest for these two companies, or any other staffing services firm also providing contracted health care providers to these two IHS facilities, to be performing accreditation compliance assessments, when part of the scope of the work is to review medical charts and provider information. Accreditation Association for Ambulatory Health Care, Inc (AAAHC) does not provide accreditation for hospitals, and therefore, would not be able to meet the needs of the hospitals that have an urgent and compelling need for onsite accreditation readiness and leadership support.

Additional Market Research Activities:

- Searched Sam.gov: using the terms “credentialing services”, “native owned”, "health management", and “health care leadership”.
- Searched GSA Advantage: using the terms: “credentialing services”, “native owned”, "health management", and “health care leadership”.

Neither search identified any responsive firms.

Native Owned Economic Entities: To identify possible responsive firms, web searches were executed to identify any firms that might be able to perform this work, conducted via Google. Search methodology: A review of native-owned firms did not identify any native-owned firms that offered medical claims analysis services. Using search terms: “credentialing services”, “native owned”, "health management", “health care leadership”, and “Independent Medical Reviews”.

A review of non-native firms identified several potential vendors, though they the capability to provide specialized and highly technical expertise was not clear from the review of publicly available information.

Other Firms:

- Small Businesses: dropping the “native owned firm”, and reviewing google searches did not identify any small business firms that offered services in this space.
- GSA Schedule: no category covering accreditation services or health facility leadership/Management was identified.

9. Procurement History:

Purchase order or contract number:

Was action competed?	☐ Yes	☒ No
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If action was not competed, state the cited authority, summarize rationale, and describe the actions that were supposed to be taken to increase competition and the results thereof:

The Hopi Health Center required immediate services due to contamination of the blood bank that closed lab services for the hospital in July 2024. The Chartis Group was the only company who was capable of performing the immediate on-site services to get the hospital back to meet CMS and accreditation requirements for quality and safe patient care. The J&A was approved by the IHS Competition Advocate under FAR 6.302-2: Unusual and compelling urgency, 41 U.S.C. 3304(a)(2).

10. Additional Information to support the justification:

If applicable, explain why technical packages or specifications to facilitate competition were not developed or available; describe actions taken to remedy the situation.

Failure to maintain accreditation and quality of care at individual facilities, will result in losing facility accreditation or the threat of loss. The utilization of this contract support will provide strategies to IHS facility leadership that will ensure no disruption is services and continue meeting IHS’s mission.